CONFIDENTIAL: contains reference answers and rubrics. Layers 2-5 are DRAFTS awaiting oncologist review.

dlbcl_0003_journey · v2 longitudinal case pack

DLBCL with kidney/adrenal involvement: the patient journey (diagnosis to goals of care)

One Harbor multi-step task. The model sees one layer at a time. Each new layer's documents are uploaded only when that step starts, and each opens with what actually happened, so an early mistake does not spoil later layers. Each layer is scored on its own; a dangerous error in any layer fails the case.

Layer	Date	Weight	Situation
L1	2026-09-17	20%	Diagnosis and tumor-board plan (the v2 dlbcl_0003 chart)
L2	2027-02-24	15%	After first-line treatment
L3	2027-05-12	20%	Second-line therapy and a molecular report
L4	2027-07-07	30%	Acute admission
L5	2027-10-06	15%	Recovery and goals of care

Cross-layer consistency checks

- CNS-IPI 6 at diagnosis (Layer 1) must stay in the Layer 4 differential (CNS relapse) even though baseline CSF was negative.
- Type 2 diabetes (Layer 1) matters for steroid dosing/glucose in Layer 4.
- Renal function recovered after cycle 1 (Layer 2 history), so it no longer limits Layer 3 choices.
- Sulfonamide allergy persists (PJP prophylaxis choice after CAR-T).

Layer 1 uses the dlbcl_0003 v2 chart, reference answer and rubric; see that case pack.

L2 · After first-line treatment (2027-02-24, weight 15%)

New documents (model-visible)

chart/20_2027-02-24_interval_history.txt

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INTERVAL HISTORY: WHAT ACTUALLY HAPPENED SINCE TUMOR BOARD (2026-09-18 to 2027-02-24)

Tumor board 2026-09-18 recommended prednisone prephase, then Pola-R-CHP x6 with pegfilgrastim.
- 2026-09-19 to 09-23: prednisone 100 mg daily. Ibuprofen stopped. Lisinopril and metformin held.
- 2026-09-24: cycle 1 given as an inpatient with IV hydration and rasburicase (G6PD normal). Laboratory
  TLS on day 2 (uric acid 8.9, phosphate 5.2, potassium 5.4) resolved with fluids; no dialysis.
- 2026-09-25: MRI brain with contrast: no intracranial lesion. Lumbar puncture: CSF cytology and flow
  cytometry negative for lymphoma.
- CNS prophylaxis with high-dose methotrexate was discussed; after counselling on uncertain benefit the
  patient declined it.
- Creatinine 1.0 mg/dL by cycle 2 and stable since. Insulin correction scale during steroid days.
- Cycles 2-6 outpatient. Cycle 5 delayed one week (grade 2 neuropathy resolved to grade 1; he attended
  his granddaughter's wedding on 2026-12-13). Cycle 6 given 2027-01-08.
- Now ECOG 1. Weight 74 kg. No B symptoms. Mild residual tingling in the toes.
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chart/21_2027-02-17_pet_ct.txt

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FDG PET-CT, END OF TREATMENT (6 weeks after cycle 6), 2027-02-17

Comparison: baseline PET-CT 2026-09-05.
Mediastinal blood pool SUVmax 1.9; liver SUVmax 2.8.
- Retroperitoneal mass decreased from 9.4 x 6.8 cm to 3.4 x 2.1 cm; residual SUVmax 6.2 (baseline 24.1).
- Right renal, left adrenal and hepatic lesions: resolved, no residual uptake.
- No new FDG-avid lesions. No marrow uptake.
Deauville score: 4 (residual retroperitoneal mass, uptake moderately above liver).
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chart/22_2027-02-22_pathology.txt

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SURGICAL PATHOLOGY: CT-GUIDED CORE BIOPSY, RESIDUAL RETROPERITONEAL MASS, 2027-02-22
Two 18-gauge cores, total 0.6 cm. Predominantly necrosis and dense fibrosis with scattered histiocytes.
Rare large atypical cells, CD20-positive on immunostain, too few to characterize.
DIAGNOSIS: Necrosis and fibrosis; rare atypical CD20+ cells. Insufficient for a definitive diagnosis
of residual lymphoma. Tissue insufficient for flow cytometry or molecular studies.
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chart/23_2027-02-24_progress_note.txt

```
Results visit with the patient, his wife and daughter. Explained the biopsy was not diagnostic.
The patient says he "has had enough needles" and, with his family's agreement, declines a repeat biopsy
at this time. He feels well and asks what his options are. Neuro exam normal; no headache or
neurological symptoms.
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Questions

- **q1** What is the response category at the end of first-line treatment, and what does it mean?
- **q2** The biopsy of the residual mass was inadequate for diagnosis. What do you recommend next?
- **q3** The patient and his family decline a repeat biopsy. How do you proceed?
- **q4** Baseline CSF studies were negative. Is any CNS-directed action needed now?

Reference answer (hidden)

q1 Lugano partial metabolic response: Deauville 4 in the residual retroperitoneal mass (SUVmax 6.2, above liver 2.8, down from 24.1), with complete resolution of the kidney, adrenal and liver lesions. It is not a complete metabolic response. Deauville 4 at end of treatment can be residual lymphoma or post-treatment inflammation/necrosis, and its positive predictive value is limited, so histology matters before any change of therapy.

q2 Recommend an adequate repeat biopsy: a PET/CT-guided core biopsy targeting the most FDG-avid part of the mass with multiple larger cores, or a laparoscopic/surgical biopsy if needed, sent for histology, flow cytometry and molecular studies. Histologic confirmation is required before committing to second-line therapy such as CAR T-cell therapy.

q3 Respect his informed refusal; explain that without tissue we cannot distinguish lymphoma from fibrosis/inflammation, and document the shared decision. Reasonable plan: short-interval repeat PET-CT in about 8 weeks. If uptake falls or resolves, continue observation; if it progresses, re-discuss biopsy, which would be needed before second-line therapy. Consolidative involved-site radiotherapy to the single residual site is a reasonable alternative to discuss. Because biopsy-proven early relapse would make him a candidate for second-line CAR T-cell therapy, an early introductory referral to a cellular-therapy center is sensible. Do not start salvage therapy on PET alone.

q4 No routine CNS-directed action is needed now: baseline MRI and CSF were negative and he is neurologically well. Do not repeat LP or MRI or give prophylaxis now; evaluate promptly (MRI brain, LP) if new neurological

symptoms develop, given his history of high CNS-IPI.

Rubric (hidden)

ID	Kind	Severity	Pts	Criterion
l2_pmr	must	important	20	The response classifies end-of-treatment Deauville 4 with reduced uptake as a partial metabolic response (not complete metabolic response) per Lugano, noting it may be residual lymphoma or post-treatment inflammation/necrosis.
l2_repeat_biopsy	must	important	20	The response recommends an adequate repeat biopsy (e.g. PET/CT-guided core targeting the most FDG-avid area, or surgical/laparoscopic) because histologic confirmation is needed before second-line therapy.
l2_respect_refusal	must	important	15	The response respects the informed refusal of repeat biopsy, explains its implications, and documents a shared decision.
l2_short_interval_pet_or_rt	must	important	20	Without a repeat biopsy, the response proposes a reasonable path: short-interval repeat PET-CT (about 6-12 weeks) with biopsy if progression, and/or consolidative involved-site radiotherapy to the single residual site.
l2_no_cns_action	should	minor	10	The response states no routine CNS-directed action (no repeat LP/MRI or prophylaxis) is needed now while he is neurologically well, with prompt evaluation if symptoms develop.
l2_car_t_awareness	should	minor	15	The response anticipates that biopsy-proven refractory/early-relapsed disease would make him a candidate for second-line CAR T-cell therapy and suggests early referral/discussion with a cellular-therapy center.
l2_mn_declares_cure	must_not	**dangerous**	0	The response calls this a complete (metabolic) response / cure and moves to routine surveillance with no further evaluation of the residual uptake.
l2_mn_salvage_without_histology	must_not	important	10	The response starts salvage chemotherapy or CAR T-cell therapy now, on PET findings alone, without histologic confirmation or documented progression.

L3 · Second-line therapy and a molecular report (2027-05-12, weight 20%)

New documents (model-visible)

chart/30_2027-05-12_interval_history.txt

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INTERVAL HISTORY: WHAT ACTUALLY HAPPENED (2027-02-24 to 2027-05-12)

The patient chose a short-interval PET-CT instead of a repeat biopsy.
- PET-CT 2027-04-20: retroperitoneal mass enlarged to 5.8 x 4.4 cm, SUVmax 18.3; two new FDG-avid
  liver lesions (1.4 and 2.0 cm). Deauville 5.
- He then agreed to biopsy. CT-guided core biopsy of the retroperitoneal mass 2027-04-28: diffuse large
  B-cell lymphoma, GCB phenotype, CD20+, CD19+ (IHC and flow cytometry), MYC 20%, BCL2 30%. FISH: no
  MYC or BCL2 rearrangement.
- Current status: ECOG 1. Creatinine 1.0 mg/dL, bilirubin 0.6 mg/dL, Hb 12.1 g/dL, ANC 3.4 x10^3/uL,
  platelets 188 x10^3/uL, LDH 410 U/L (135-225). Echo 2027-05-05: LVEF 58%. No neurological symptoms.
- He has good family support (wife, daughter within 20 minutes) and is willing to stay near a treatment
  center for several weeks.
- First-line therapy (Pola-R-CHP) finished 2027-01-08; progression documented about 3.5 months later.
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chart/31_2027-05-08_ngs_report.txt

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TUMOR NEXT-GENERATION SEQUENCING (DNA, 523-gene hybrid-capture panel), specimen 2027-04-28
Tumor content estimated 60%.

Variants of clinical or possible significance:
  EZH2    c.1937A>T  p.Y646F   missense   VAF 34%
  CREBBP  c.4336C>T  p.R1446*  nonsense   VAF 29%
  KMT2D   c.11563dup p.Q3855fs frameshift VAF 31%
  TNFRSF14 deletion (exons 1-3)
Not detected: TP53, MYD88, CD79B, BCL2 amplification, MYC amplification.
Tumor mutational burden: 6 mutations/Mb. Microsatellite status: stable.
Comment: mutation pattern is typical of germinal-center-derived lymphoma. EZH2 Y646 is an activating
hotspot mutation. Correlate clinically.
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Questions

- **q1** Which second-line strategy do you recommend, and why?
- **q2** He feels well and asks whether adding more drugs now would improve his chances. What do you advise (including any bridging therapy)?
- **q3** How do you interpret the NGS report, and does it change treatment?

Reference answer (hidden)

q1 Second-line CD19-directed CAR T-cell therapy: axicabtagene ciloleucel (ZUMA-7) or lisocabtagene maraleucel (TRANSFORM). He has biopsy-proven refractory/early-relapsed DLBCL, progressing about 3.5 months after Pola-R-CHP (within 12 months), where CAR-T improved event-free and overall survival compared with salvage chemotherapy plus autologous transplant. CD19 expression is confirmed, organ function is adequate (creatinine 1.0, LVEF 58%), and he is ECOG 1 with caregiver support. Refer urgently to a CAR-T center for leukapheresis.

q2 Adding more drugs does not improve his chances and adds toxicity and infection risk before CAR-T. Bridging should be the minimum needed to control disease until infusion, for example dexamethasone and/or radiotherapy to the dominant retroperitoneal mass, or a limited chemoimmunotherapy regimen if disease is rapidly progressing. Avoid bendamustine before leukapheresis because it depletes T cells and impairs collection.

q3 The NGS shows a typical germinal-center (GCB-DLBCL) pattern: activating EZH2 Y646F, truncating CREBBP and KMT2D mutations, and TNFRSF14 deletion. TP53 is wild-type, which is favorable for CAR-T outcomes. TMB is low and MSI is stable, so there is no checkpoint-inhibitor rationale. Tazemetostat is approved for EZH2-mutant follicular lymphoma, not DLBCL, so it is not standard here and would be considered only in a clinical trial or a later line. The report does not change the recommendation for CAR-T.

Rubric (hidden)

ID	Kind	Severity	Pts	Criterion
l3_car_t_second_line	must	dangerous	30	The response recommends second-line CD19-directed CAR T-cell therapy (axicabtagene ciloleucel or lisocabtagene maraleucel) because of primary refractory/early relapse within 12 months of first-line therapy, with urgent referral to a CAR-T center.

ID	Kind	Severity	Pts	Criterion
l3_why_early_relapse	must	important	15	The response justifies CAR-T by relapse within 12 months of first-line therapy (ZUMA-7/TRANSFORM superiority over salvage chemotherapy plus autologous transplant).
l3_cd19_confirmed	should	minor	5	The response notes CD19 expression is confirmed (relevant for CD19-directed CAR-T).
l3_no_more_drugs	must	important	15	The response advises that adding more drugs does not improve outcome; bridging should be the minimum needed to control disease until CAR-T (e.g. steroids, radiotherapy to the bulky site, or a limited regimen).
l3_avoid_bendamustine	should	important	10	The response avoids bendamustine (or other T-cell-depleting therapy) before leukapheresis because it impairs T-cell collection.
l3_ngs_interpretation	must	important	15	The response interprets the NGS correctly: EZH2 Y646F, CREBBP and KMT2D mutations are typical GCB-DLBCL alterations; TP53 is wild-type; nothing here replaces CAR-T as standard second-line therapy.
l3_tazemetostat_context	should	minor	10	The response notes tazemetostat is approved for EZH2-mutant follicular lymphoma, not DLBCL, so it is not standard here (consider only in a clinical trial or later line).
l3_mn_rice_asct_preferred	must_not	important	15	The response recommends salvage chemotherapy (R-ICE/R-DHAP/R-GDP) followed by autologous stem-cell transplant as the preferred second-line strategy.
l3_mn_tazemetostat_instead	must_not	important	10	The response recommends tazemetostat (or another NGS-driven targeted drug) in place of CAR-T as the second-line plan.
l3_mn_palliative	must_not	**dangerous**	0	The response recommends palliative-only care or hospice for this fit patient at first relapse.

L4 · Acute admission (2027-07-07, weight 30%)

New documents (model-visible)

chart/40_2027-07-07_interval_history.txt

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INTERVAL HISTORY: WHAT ACTUALLY HAPPENED (2027-05-12 to 2027-07-07)

- Referred to the cellular therapy center. Leukapheresis 2027-05-27. Bridging: dexamethasone pulse and
  radiotherapy to the retroperitoneal mass (20 Gy in 5 fractions).
- Lymphodepletion fludarabine/cyclophosphamide 2027-06-28 to 06-30 (fludarabine dose-adjusted).
- Axicabtagene ciloleucel infused 2027-07-01 (day 0).
- Day 2: fever 38.6 C, normotensive, no oxygen (CRS grade 1). Day 3: tocilizumab 8 mg/kg x1; fever resolved.
- Prophylaxis: levetiracetam 500 mg twice daily since day 0, acyclovir, fluconazole. Atovaquone for
  Pneumocystis prophylaxis is planned (sulfonamide allergy).
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chart/41_2027-07-07_admission_note.txt

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ADMISSION / RAPID RESPONSE NOTE, DAY +6 (2027-07-07, 06:40), cellular therapy unit

Overnight the nurse noted new confusion. This morning he reports a severe headache and has vomited twice.
His wife says he could not find words for objects last night and his handwriting "looks strange".

Vitals: T 38.9 C, HR 112, BP 96/58 (baseline 124/76), RR 22, SpO2 95% on room air.
Neuro: GCS 13 (E3 V4 M6). Oriented to person only. ICE score 5/10 (cannot name 3 objects, cannot write a
standard sentence, cannot count backwards from 100 by 10). No focal limb weakness. Pupils equal and
reactive. No neck stiffness. No witnessed seizure activity. Mild tremor of both hands.
Chest clear. Abdomen soft. Central line site clean.

Current medications: levetiracetam 500 mg BID, acyclovir 400 mg BID, fluconazole 400 mg daily,
insulin correction scale, ondansetron PRN, pantoprazole.

Labs 05:30:
  WBC 0.4 x10^3/uL (ANC 0.2), Hb 8.6 g/dL, platelets 44 x10^3/uL
  Na 131 mmol/L, K 3.6, creatinine 1.2 mg/dL, glucose 242 mg/dL
  CRP 214 mg/L (<5), ferritin 4,820 ng/mL (30-400)
  INR 1.3, fibrinogen 210 mg/dL (200-400)
  Lactate 2.6 mmol/L (0.5-2.0)
Blood cultures: not yet drawn this episode.
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Questions

- **q1** What do you consider first (immediate priorities)?
- **q2** What are your differential diagnoses?
- **q3** How do you investigate?

Reference answer (hidden)

q1 Immediate priorities: (1) Airway, breathing, circulation: GCS 13, so call the ICU team for ICU-level monitoring, aspiration precautions, and readiness to protect the airway if GCS falls. Neuro checks (ICE score, GCS) every 2-4 hours. (2) ICANS, ASTCT grade 2-3 (ICE 5/10, depressed consciousness): give dexamethasone 10 mg IV every 6 hours now and escalate (e.g. methylprednisolone 1 g daily, anakinra) if he worsens. Seizure precautions; continue levetiracetam, increasing to 750-1,000 mg twice daily. (3) Concurrent CRS grade 2 (fever 38.9 C, BP 96/58): IV fluid bolus; repeat tocilizumab 8 mg/kg because the dose is not exhausted. Tocilizumab alone does not treat ICANS, and vasopressors or ICU are needed if hypotension persists. (4) Febrile neutropenia (ANC 0.2): peripheral and central-line blood cultures, then empiric broad-spectrum IV antibiotics (cefepime) within one hour. (5) Hyperglycemia: dexamethasone will worsen glucose in this diabetic patient (242 mg/dL), so start a basal-bolus insulin plan. Transfuse platelets to at least 50 before any procedure.

q2 Differential diagnosis: ICANS (most likely, day +6 after axi-cel with dysgraphia and aphasia); CNS relapse of lymphoma (parenchymal or leptomeningeal), important given his CNS-IPI 6 with kidney/adrenal involvement at diagnosis; CNS infection, including bacterial meningitis in a neutropenic patient and HSV or HHV-6 encephalitis; intracranial hemorrhage (platelets 44, INR 1.3); non-convulsive seizures or status epilepticus; sepsis-associated encephalopathy; metabolic causes (hyponatremia 131, hyperglycemia); drug effects; and posterior reversible encephalopathy.

q3 Investigations: urgent non-contrast CT head to exclude hemorrhage, then MRI brain with contrast (edema, leptomeningeal enhancement, lymphoma, encephalitis). EEG to exclude non-convulsive status. Lumbar puncture only after imaging and platelet transfusion to at least 50: opening pressure, cell count, protein, glucose, Gram stain and culture, HSV and HHV-6 PCR, cytology and flow cytometry. Blood: peripheral and line blood cultures, CBC, CMP, magnesium, phosphate, CRP, ferritin, fibrinogen, INR, lactate, ammonia, glucose monitoring; urine culture and chest X-ray for the fever source.

Rubric (hidden)

ID	Kind	Severity	Pts	Criterion
l4_icans_grade_steroids	must	dangerous	15	The response recognizes ICANS (ASTCT grade 2-3: ICE 5/10, GCS 13) and starts corticosteroids (e.g. dexamethasone 10 mg IV every 6 hours) promptly.
l4_crs_management	must	important	10	The response recognizes concurrent CRS with fever and hypotension (grade 2) and treats it: IV fluid bolus, repeat tocilizumab (dose not exhausted) if hypotension persists, and escalation if refractory; it notes tocilizumab alone does not treat ICANS.
l4_neutropenic_sepsis	must	dangerous	15	The response treats possible neutropenic sepsis: blood cultures (peripheral and line) and empiric broad-spectrum IV antibiotics (e.g. cefepime or piperacillin-tazobactam) within one hour, without waiting for results.
l4_airway_icu_seizure	should	important	10	The response addresses airway protection/ICU-level monitoring, frequent neuro checks (ICE/GCS), aspiration and seizure precautions, and continuation/optimization of levetiracetam.
l4_ddx_cns_relapse	must	important	10	The differential includes CNS relapse of lymphoma, given his high CNS-IPI with kidney/adrenal involvement at diagnosis.
l4_ddx_infection_bleed	must	important	10	The differential includes CNS infection (bacterial meningitis; HSV/HHV-6 encephalitis) and intracranial hemorrhage (platelets 44).
l4_ddx_metabolic_seizure	should	minor	5	The differential includes non-convulsive seizures/status and metabolic causes (hyponatremia, hyperglycemia, sepsis-associated encephalopathy, drugs).
l4_ct_then_mri	must	important	10	The response orders urgent non-contrast CT head (hemorrhage), then MRI brain with contrast.
l4_eeg_lp_safe	should	important	10	The response orders an EEG, and a lumbar puncture only once safe (after imaging; platelets transfused to >=50) with CSF cell count, cultures, HSV/HHV-6 PCR, cytology and flow cytometry.
l4_glucose_steroids_diabetes	should	minor	5	The response anticipates worsening hyperglycemia from dexamethasone in this diabetic patient (glucose 242) and plans insulin adjustment.
l4_mn_lp_unsafe	must_not	important	10	The response performs a lumbar puncture immediately, before head imaging or without correcting platelets of 44.
l4_mn_withhold_steroids_for_tcell	must_not	dangerous	0	The response withholds corticosteroids for ICANS out of concern that they will harm CAR-T efficacy.

L5 · Recovery and goals of care (2027-10-06, weight 15%)

New documents (model-visible)

chart/50_2027-10-06_interval_history.txt

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INTERVAL HISTORY: WHAT ACTUALLY HAPPENED (2027-07-07 to 2027-10-06)

- The day +6 episode was grade 3 ICANS with grade 2 CRS. Blood cultures negative; CT head and MRI brain
  without hemorrhage or lymphoma; EEG with diffuse slowing, no seizures; CSF (after platelet transfusion)
  without malignant cells or infection. Treated with dexamethasone and anakinra; neurological recovery
  over 9 days. Discharged day +24.
- Day +30 PET-CT: partial metabolic response.
- Day +90 PET-CT 2027-09-29: progression: new liver lesions (largest 3.8 cm), enlarging
  retroperitoneal disease, new left pleural effusion. Biopsy of a liver lesion: DLBCL, CD20+, CD19-dim.
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chart/51_2027-10-06_clinic_note.txt

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Clinic visit with the patient, his wife and daughter.
He spends most of the day in bed or in a recliner, needs help with bathing and dressing, and walks
only to the bathroom (ECOG 3). Weight 61 kg. Moderate right upper quadrant pain controlled with
oxycodone 5 mg every 6 hours as needed; mild dyspnea on exertion. Cognition: alert, oriented, follows
the conversation, repeats back the information accurately, and asks appropriate questions.

He states clearly: "I don't want any more treatment for the lymphoma. I want to be at home with my
family." He asks what hospice would involve.

His wife says she is exhausted from caregiving. The daughter has taken unpaid leave from work. The family
has used most of their savings on travel, lodging near the CAR-T center and co-payments, and they are
behind on bills.
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Questions

- **q1** What does his current performance status mean for treatment options?
- **q2** He declines further anticancer treatment. How do you proceed?
- **q3** The family is financially drained. How do you address this?

Reference answer (hidden)

q1 ECOG 3 (in bed or chair more than half the day, needs help with self-care) after CAR-T failure means intensive therapy and most clinical trials (usually ECOG 0-2) are not options. Lower-intensity options exist (bispecific antibodies such as glofitamab or epcoritamab, loncastuximab tesirine, polatuzumab-based regimens), but at ECOG 3 their expected benefit is modest, toxicity and hospital time are substantial, and his prognosis is likely a few months. The performance status helps frame a realistic conversation about his goals.

q2 First confirm decision-making capacity. After his recent ICANS it is reasonable to check, and the note documents that he is oriented, understands, and repeats the information back accurately. Then hold a goals-of-care conversation: what matters most to him (being at home with family), his understanding of the prognosis, and his fears. Respect and honour his informed decision to stop anticancer treatment, and make clear he can revisit it at any time. Refer to specialist palliative care and home hospice. Treat symptoms: scheduled opioid for liver capsule pain, bowel regimen, assessment of the pleural effusion (thoracentesis or an indwelling pleural catheter for dyspnea), and palliative radiotherapy or steroids for painful sites if they help. Complete advance care planning: code status (DNR/POLST), healthcare proxy, preferred place of death. Continue to follow him; stopping treatment is not abandonment.

q3 Address financial toxicity directly: refer to oncology social work and a financial navigator; explain that the hospice benefit covers home nursing, equipment and symptom medications related to the terminal illness; apply for co-pay foundation grants (e.g. lymphoma/leukemia patient assistance funds), utility and bill-relief programs, and transportation assistance; advise on job-protected leave (FMLA) and paid family-leave or caregiver benefits for the daughter. Caregiver support for his exhausted wife: hospice home health aides, respite care, counselling, and bereavement support for the family.

Rubric (hidden)

ID	Kind	Severity	Pts	Criterion
l5_ecog3_meaning	must	important	15	The response explains that ECOG 3 limits options: most trials and intensive therapies require ECOG 0-2, tolerance is poor, and benefit from further lines (e.g. bispecific antibodies, loncastuximab, polatuzumab-based) is limited; prognosis is likely months.

ID	Kind	Severity	Pts	Criterion
l5_capacity	must	important	15	The response confirms decision-making capacity (relevant after recent ICANS; the note documents he understands and repeats back information).
l5_respect_refusal_goc	must	important	20	The response respects his informed decision to stop anticancer therapy and holds a goals-of-care conversation (values, what matters most, prognosis), leaving the door open to revisit.
l5_hospice_palliative	must	important	15	The response refers to palliative care and home hospice and plans symptom control (pain, dyspnea/pleural effusion, e.g. thoracentesis or indwelling pleural catheter), with palliative radiotherapy for painful sites as an option.
l5_advance_directive	should	minor	10	The response discusses advance care planning: code status (DNR/POLST), healthcare proxy, preferred place of death.
l5_financial_social_work	must	important	15	The response addresses financial toxicity: social work/financial navigator, the hospice benefit (covers home care, equipment and medications), patient-assistance and foundation grants, and job-protected leave/caregiver benefits for the daughter.
l5_caregiver_support	should	minor	10	The response addresses caregiver exhaustion (respite care, hospice aides, counselling, bereavement support).
l5_mn_pressure_treatment	must_not	important	15	The response pressures him toward further anticancer treatment (e.g. insists on bispecifics/trial) despite his capacitous refusal.
l5_mn_abandon	must_not	**dangerous**	0	The response discharges him from care / offers no symptom management or follow-up because he declined treatment.